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Name: نورا نجاح عبد العزيز فوج National ID: 28006012104662	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
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المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Zoledronic acid for bone protection during hormonal therapy
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية الكويت العربية
مملكة نفط النسيج

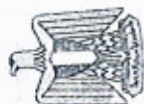


نوره
نجاح عبدالعزيز فرج محمد بدير
قرية البراجيل
مركز أوسيم - الجيزة



٢٨٠٠٦٠١٢١٠٤٦٦٢

KW8978860



٢٠٢٤/١٢ ٢٨٠٠٦٠١٢١٠٤٦٦٢
حاصله على دبلوم ادارة وسكرتاريه
انثى مسلمة
محمد محمد جاب الله احمد
البطاقة سارية حتى ٢٠٣١/١٢/٣٠
متروجة





المرأة الأولى / ٤٦٦٥ / ١٥١٠١٠٢٠٢٠٢

١١٥٧٩٥١٨٥٥ / ٥

وزارة الصحة والمساكن
المراكز الطبية المتخصصة
مركز أورام دار السلام
بغداد، العراق

Name : نورة جعفر

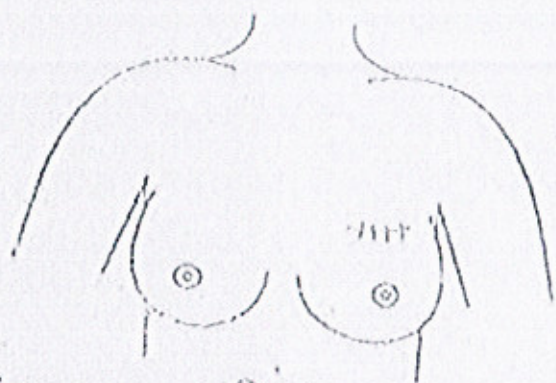
Age : ٤١

No. : ١١٦٥ / ٥٤٤

15-4/2021

MDT FORM

Clinical data :



lt breast excision IDC G.U.
free surgical margin
T1C

Reason of discussion:

ER	+	7/8
PR	+	4/8
HER2	-	ve

MDT Outcome and Plan

- ① THP needed
- ② SNL (SNB)

[20/5/2021] SNB of axillary pathology The path is final
Signature : [Signature]
[Signature]

Name: نورا نجاح عبد العزيز فرج		GUSTAVE ROUSSY	
MR. : 2410			
Detailed Medical Report			
			السيد
تحتية طبية وبعد,,			
		إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
		رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته			
تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	
وبعد الكشف والفحوصات والمتابعة وجد انه			
8/1/2026 MDT 13/12/2025 FOR PORT AI+ZOLADEX CDK46 ZOLADEX AT 25/12/2025 FEMARA CALCIUM 1*1*90 ONE ALPHA PORT REPORT DONE REPORT FOR CDK46 FROM NGO E2 30 FASH 15.3 FOR ZOLADEX 10.8 SC 46-year-old female, ER 7/8, PR 4/8, HER2 negative, T1N0 breast cancer diagnosed 2021. Underwent It BCS followed by adjuvant chemotherapy and PORT. Adjuvant Endocrine Therapy: · Started Nolvadex + Zoladex from August 2022. · Tamoxifen continued until November 2025. Recent Follow-up & Procedures: · Transvaginal ultrasound: bulky uterus with endometrial thickening. · D&C (Dec 2025): erosions, acute inflammatory process with polyp formation, repetitive squamous atypia, and tiny endometrial fragments showing early hyperplasia without atypia. · Bilateral sonomammography (Oct 2025): right breast BI-RADS 3, upper outer quadrant, probably benign lesion, follow-up in 6 months. patient shifted to zoladex , femara 01/2026 patient present today for zoladex FEMARA + SUPP >> 3M BIL U\S >> R ZOLADEX ON 18\4\2026 4/2026SM :RT BREAST BIRADS 3+LT BREAST BIRADS 2 TODAY FOR :ZOLADEX			
تحريراً في ,			
		التاريخ :	19-09-2026
		الوقت :	
		إيه مدحت بدوي محمد	الطبيب المعالج :
يعتمد ,			



وزارة الصحة
المملكة العربية السعودية

Pathology Report

Name: نوره نجاة عبد العزيز Age: 40 years.

Clinical data: Left breast WLE invasive duct carcinoma grade II.

Specimen type: Left breast sentinel lymph nodes.

Date (Received): 18 /4/2021

Date (reported): 5/5/2021

Lab. Number: 844 /21

Hospital Number:

Gross Description:

Received for frozen section: A fatty tissue piece measuring 8x1cm entangling soft rubbery 7 lymph nodes ranging from 0.3x0.3cm up to 1x1cm.

Microscopic picture:

Sections examined reveals preserved normal nodal architecture. The subcapsular sinuses are patent. The lymph follicles are hyperactive and have prominent germinal centers. There is no evidence of specific granuloma or neoplasia in sections examined.

Diagnosis:

Left breast sentinel lymph nodes.

Lymph nodes with reactive changes.

Negative for metastatic deposits (0/7).

Prof. Dr. Lubna Omar.
Dr. Abdulrahman Ibrahim.

Dr.
Elia Anis Ishak

M. Sc. Pathology - Prof. of Pathology, Cairo University
Fellow, Armed Forces Institute of Pathology, USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis

M. Sc. Pathology, Ass. Prof. of Pathology, Cairo University
Fellow, Mass General Hospital, Harvard, USA
Member, United States Canadian Academy of Pathology

Patient Name : نوره نجاح عبد العزيز

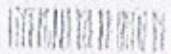
Age : 40 Y

Referred By Prof. Dr. : د. محمد احمد عزيز

Clinical Diagnosis : Left breast mass

Nature of Specimen : Excision

Receiving Date : 15/03/2021

Analysis ID :  537513

Gender : Female

Patient ID : 422086

PATHOLOGY REPORT

Gross :

Excision received consists of a piece of yellowish breast tissue measured 8x7x4.5 cm, covered by skin ellipse measured 4x1 cm. Cut section revealed firm gritty tumor with ill defined borders, measured 1.2x0.8x0.5 cm.
Free least surgical margin measured 0.3 cm

Histoscopic :

Sections examined from the specimen received revealed breast tissue showing tumor composed of irregular groups of malignant ductal cells showing moderate nuclear pleomorphism and hyperchromasia.

There is poor attempt at tubular formation.

There is moderate number of mitotic figures.

There is moderate desmoplasia.

There is peritumoral lymphocytic infiltration around 10%.

Lympho-vascular invasion is Present.

There is necrosis.

There is an intraductal component about 10% formed of dilated ducts with malignant lining showing solid pattern with central necrosis.

The surrounding breast tissue showed fibroelastosis.

The skin is free.

Surgical margins are free.

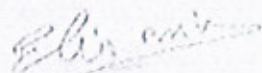
Diagnosis :

Left breast mass, Excision, INFILTRATING DUCT CARCINOMA GRADE II, INTRADUCTAL COMPONENT 10%, FREE SURGICAL MARGINS.

FIGO STAGE: T1c.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak



For any further queries, please contact the laboratory at 0111 2311 1111 or 0111 2311 1112.

1/1

مركز تشخيص - باورل - ميدان يحيى - الدور الخامس و السابع

0111 2311 1111 - 0111 2311 1112 - 0111 2311 1113



Patient Name: Norah Nagah Abdel Aziz Farag	Date : 09/10/2024
Exam : MAMMOGRAPHY BOTH	Age : 44 Year(s)
Medical ID: 1165/2021	Pat. ID: 13640

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Follow up.

Report:

- ACR:8.

Right breast:

- No suspicious mass lesion or architectural distortion>>> confirmed by US.
- UOQ Intramammary lymph node confirmed by US, the above mentioned lesion about 2 cm from it.
- Right axillary non specific lymph node.

Left breast:

- Left breast UOQ focal area of parenchymal distortion owing to prior surgical interference, with post-operative scarring and diffuse edema pattern in the form of increased trabeculations and skin thickness.
- Complementary ultrasound showed left breast UOQ focal parenchyma distortion, owing to prior surgical interference with clear operative bed with no evidence of recurrent mass lesions.
- Single prominent left level I lymph node measuring 1.2X0.7 cm.
- Free left Infra and supra clavicular regions.

Opinion:

- Left breast focal parenchymal distortion owing to prior surgical interference (CBS) with post-operative sequel, with free operative bed with no evidence of recurrent mass lesions (BIRADS 2)
- Left axillary prominent lymph node for close follow up.
- Right breast benign finding (BIRADS 2).

Signature Dr. Nehal Hamdy
09/10/2024 1:05 PM